



GOLD MEDAL INJURY LAW

WASHINGTON

253-215-1271

7602 BRIDGEPORT WAY W, SUITE 3B
LAKEWOOD, WASHINGTON 98499

FAX: (877) 813-6042

NEVADA

702-304-7625

2001 S RAINBOW BLVD, SUITE 140
LAS VEGAS, NEVADA 89146

FAX: (702) 442-1852

{{TODAY_LONG}}

SENT VIA FIRST CLASS MAIL, FASCIMILE AND EMAIL

{{insurance2.insurer.name}} Attn: {{insurance2.adjuster.name}}
{{insurance2.insurer.address1}}
{{insurance2.insurer.address1city}}, {{insurance2.insurer.address1state}}
{{insurance2.insurer.address1zip}}
F: {{insurance2.adjuster.fax1}}
E: {{insurance2.adjuster.email1}}

Re: Our Client: {{fullname}} and {{insurance2.p1.name}}
Your Insured: {{insurance2.insured.name}}
Claim No.: {{insurance2.claimnumber}}
Date of Loss: {{intake.incidentDate| longDate}}

Dear Mr./Ms. {{insurance2.adjuster.lastname}}

In response to your most recent offer to settle of \$ _____ and \$ _____, please allow this letter to serve as confirmation of our client's/clients' declination. Liability against your insured, who failed to stop in time and crashed into our client's/clients' car, is clear. Causation is also clearly demonstrated by the fact that our client/clients has/have never complained of the injuries referenced in his/her/their demand prior to {{intake.incidentDate| longDate}}.

At the time of accident, my client was braced for the impact with his/her head facing straight forward while his/her body jerked forward abruptly and back into the seat. Immediately after the impact, he/she experienced neck pain, mid back pain, low back pain, left shoulder and arm pain, insomnia, concussion, muscle spasms, shooting pain down the right hip, and headaches. He/she felt being dazed and the symptoms increased from the time of the accident and got significantly worse.

Your insured's negligence, as indicated by the facts of the case, caused this accident that resulted in serious medical issues that continue to plague my client to this day. Here is a summary of our client's/clients' treatment:

Mr./Ms. {{lastname}} originally treated at Chiropracted for 36 visits. Mr./Ms. {{lastname}} complained of neck, mid back and low back pain which increased with prolonged travelling, lifting, walking and standing. He/She states that the pain also increased when doing activities related to personal care such as washing or getting dressed. Dr. Teddy Sims diagnosed him/her with the following:

1. Post-concussion syndrome
2. Post traumatic headaches
3. Cervical sprain/strain with cervical segmental dysfunction
4. Thoracic spine sprain/strain with thoracic segmental dysfunction
5. Lumbar spine sprain/strain with lumbar segmental dysfunction.
6. Right hip sprain/strain
7. Muscle spasm
8. Insomnia secondary to pain.

The following forms of therapy were used:

1. Chiropractic spinal manipulation (3-4 regions)
2. Therapeutic exercise to develop strength, endurance, and flexibility
3. Hot/cold packs



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4. Electrical muscle stimulator
5. ADL Training

Mr./Ms. {{lastname}} later seen by Dr. Neville Campbell where he/she was diagnosed with the following:

1. Neck Pain
2. Low Back Pain
3. Myalgia
4. Cervicalgia – Neck Pain
5. Cervical Radiculopathy
6. Lumbar Radiculopathy

He /She was prescribed pain medications for pain and muscle spasms and was recommended to continue chiropractic care. Mr./Ms. {{lastname}} was also referred out for a MRI of the cervical and lumbar spine.

The Cervical MRI revealed the following injuries:

1. Disc protrusion at C4-C5 (4mm)
2. Disc protrusion at C5-C6 (2 to 3mm)
3. Mild right-sided neural foraminal narrowing at C4-C5 and C5-C6
4. Central spinal canal stenosis at C4-C5 with mild cervical cord impingement

These bulges caused severe pain on his/her neck that radiates to his/her left shoulder and arms. The Lumbar MRI revealed the following injuries:

1. Disc bulges at L4-L5 (1 to 2mm)

These back injuries caused mid and low back pain that extended to his/her right hamstring/hip.

Due to his/her injuries, Mr./Ms. {{lastname}} received the following injection on August 30, 2017 performed by Dr. Neville Campbell, M.D. and Ruben Alexander M.D. at Red Rock Surgery Center.

1. Left C4-C5 Cervical Transforaminal Epidural Steroid Injection
2. Left C5-C6 Cervical Transforaminal Epidural Steroid Injection
3. Fluoroscopic needle localization/guidance and Epidurogram

Post-operative diagnosis was RADICULOPATHY, CERVICAL REGION (ICD 10-M54.12-723.4). Client was discharged without complication or incident. My client describes pain as aching, sharp, dull, stiff, shooting, tiring/exhausting, throbbing and tingling with pain intensity of 5-6 that occurred 80% to 100% of the time. The pain was so severe in her neck that it sent shooting pains to her face and molars and caused paresthesias in the left side of her face. Thea pain was aggravated by bending, twisting, straining, coughing, and lifting while her headache was aggravated by stress and pain. Pain was so severe that it prohibits activity thus our client had problems with general activity, travelling, lifting, walking, standing, washing, getting dressed, enjoyment of life, mood, normal work, recreational activities, relationships with people, and sleep have all been negatively affected by this pain. He/She was not able to perform normal work activities because of pain and stress. Please take into consideration that my client has sustained these life-changing injuries because of this accident.



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Taking **Mr./Ms. {{lastname}}**'s injuries and damages into consideration, your policy limits are highly inadequate in comparison to other verdicts and settlements for similar injuries. Below are examples of other verdicts:

1. **Austin vs. Contrast Lighting Services Inc- Verdict - \$650,000.00** - MRI of the cervical spine showed C5-C6 central disc bulge **resulting from a car wreck.**
2. **Toney vs. Magana- Verdict - \$500,000.00** a lumbar MRI was the most significant evidence for a disc bulge at C4-C5, which was the basis for the verdict.

Those cases both involved Plaintiffs that suffered from only one level of disc injuries. Unlike the above cases, **Mr./Ms. {{lastname}}** has injuries at multiple disc levels, **him/her damages are therefore greater and would foreseeable lead to larger verdicts than those seen above.**

Our **client/clients is/are** understandably upset at this whole ordeal and would like to put this behind **him/her/them** and move on with **his/her/their life/lives**. This is a case where the damages will far exceed the policy limit. We will file suit against your insured and obtain a judgment far in excess of the policy limit. A jury would not hesitate to award money damages in the \$500,000.00 to \$650,000 range based on the above facts. We do not believe a jury will think your insured's failure to maintain due care and stop in time, impacting our client's vehicle was a small matter.

They will not think that failing to compensate **Mr./Ms. {{lastname}}** for **his/her/their** demonstrable injuries is okay and will award a high amount of compensatory damages here. The entire story will be heard by the jury.

We demand your \$30,000.00 which our client has expressly authorized me to accept within the next seven days. We are extending you until 5:00 p.m. April 16 to accept our offer.

Our preference is to amicably resolve this case for the policy limits without the need for protracted litigation and without subjecting your insured to an excess verdict, but if a reasonable offer is not presented, I am prepared to file on behalf of our client in the next two weeks should you choose to expose your insured to the litigation process. Thank you for your immediate attention to this matter.

Very Truly Yours,

Andrew Stikovic

{{ATTORNEY_NAME}}, ESQ.
{{FIRM_NAME}}

AS/